

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title: A Phase 1, Open-label, Multicenter Study of BMS-986360/CC-90001 Alone and in Combination With Chemotherapy or Nivolumab in Advanced Solid Tumors **Short Title/Acronym:** A Study of BMS-986360/CC-90001 Alone and in Combination With Chemotherapy or Nivolumab in Advanced Solid Tumors **Protocol Number:** IM043-004 **NCT Number:** NCT05625412 **Posting ID:** 560331813738566651 **Sponsor/Company Name:** Bristol Myers Squibb **Trial Status:** COMPLETED **Phase of Development:** PHASE 1 **Medical Monitor:** Bristol-Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Summary: The aim of this study is to assess the safety and tolerability of BMS-986360 as monotherapy and in combination with chemotherapy or nivolumab in participants with advanced solid tumors.

2.2 Background & Rationale There is a continuing medical necessity to find effective treatments for advanced solid tumors that have become resistant or refractory to standard therapies. BMS-986360 is a novel investigational agent being tested for safety and efficacy. Nivolumab and chemotherapies (like Docetaxel and Capecitabine) have established roles in managing advanced cancers (such as NSCLC and TNBC). This study evaluates if combining BMS-986360 with these established agents can improve safety profiles and patient outcomes for populations with unmet medical needs.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Dose escalation/expansion) **Blinding:** Open-label **Randomization:** Non-randomized / N/A (treatment group assignment is open-label) **Study Status:** COMPLETED

3.2 Planned Enrollment & Duration Targeted Enrollment: 70 participants **Number of Sites:** Multicenter (Global, including USA, ARG, AUS, CAN, ESP, FRA, ITA)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participants in Part 1 must have histologic or cytologic confirmation of non-small cell lung

cancer (NSCLC), metastatic triple negative breast cancer (mTNBC), squamous cell carcinoma of head and neck (SCCHN), pancreatic adenocarcinoma (PAAD), renal cell carcinoma (RCC), microsatellite-stable colorectal carcinoma (MSS CRC), or sarcoma, that is advanced (metastatic, recurrent, and/or unresectable) with measurable disease per RECIST v1.1. In Part 2, only participants with histologic confirmation of advanced NSCLC or mTNBC with measurable disease per RECIST v1.1 are eligible. 2. In Part 2, archival biopsy collected within 3 months of screening with no intervening therapy (formalin-fixed, paraffin embedded [FFPE] blocks or a minimum of 20 freshly cut unstained FFPE slides with an associated pathological report) or fresh biopsy collection at Screening and fresh biopsy collection at cycle 3 day 1 (C3D1) ($\pm$ 5 days) are mandatory, while it is strongly encouraged but optional at progression. Therefore, the participant in Part 2 must have a suitable tumor lesion for the biopsy procedure, as judged by the investigator, in order to be eligible for the study. 3. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 or 1. 4. Participants resistant/refractory to or intolerant of existing standard therapies known to provide clinical benefit (in addition, participants with NSCLC must be resistant or refractory to anti-PD-(L)1-based immunotherapy).

4.2 Exclusion Criteria 1. Participants with primary central nervous system (CNS) disease, or tumors with CNS metastases as the only disease site, will be excluded. Participants with controlled brain metastases, however, will be allowed to enroll. Controlled brain metastases are defined as no radiographic progression for at least 4 weeks following radiation and/or surgical treatment (or 4 weeks of observation if no intervention is clinically indicated), no longer taking steroids for at least 2 weeks prior to first dose of study intervention, and with no new or progressive neurological signs and symptoms. 2. Participants with a condition requiring systemic treatment with corticosteroids (> 10 mg daily prednisone equivalent) within 14 days or other immunosuppressive medications within 30 days of randomization. Inhaled or topical steroids and adrenal replacement steroid doses > 10 mg daily prednisone equivalent are permitted in the absence of active autoimmune disease. 3. Participants with concurrent malignancy or history of prior malignancy active within 2 years (except history of early-stage basal/squamous cell skin cancer or non-invasive or in situ cancers who have undergone definitive treatment) are excluded unless treatment was completed at least 2 years before randomization and the participant has no evidence of disease. 4. Participants with NSCLC with known or not tested for epidermal growth factor receptor (EGFR) or V-raf murine sarcoma viral oncogene homolog B1 (BRAF) V600E mutations, or anaplastic lymphoma kinase (ALK) or receptor tyrosine kinase (ROS1) translocations sensitive to available targeted inhibitor therapy. *Note: Other protocol-defined inclusion/exclusion criteria apply.*

5. Intervention & Treatment Plan

5.1 Investigational Regimen Investigational Products: BMS-986360 (CC-90001), Capecitabine, Nivolumab, and Docetaxel. (*Docetaxel details: ATC Code: L01CD02 | Trade Names: Docefrez, Taxotere | EMA Information: [Docefrez EPAR](#), [Taxotere EPAR](#)*) **Dosage/Frequency:** Escalating dose levels of BMS-986360 alone or combined with standard doses of nivolumab, docetaxel, or capecitabine depending on the assigned cohort. **Route of Administration:** Intravenous (IV) and Oral (PO). **Duration of Treatment:** Up to approximately 2 years.

5.2 Concomitant & Prohibited Therapy Permitted: Inhaled or topical steroids and adrenal replacement steroid doses > 10 mg daily prednisone equivalent are permitted in the absence of active autoimmune disease. **Prohibited:** Systemic treatment with corticosteroids (> 10 mg daily prednisone equivalent) and other immunosuppressive medications during specific pre-treatment washout windows.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline Imaging (RECIST v1.1), Archival/Fresh Biopsy collection
Baseline	Day 1	Cycle 1 Day 1 Dosing, Baseline Safety Labs, PK/PD Blood Sampling
Interim	Every Cycle	Safety Labs, Efficacy Assessment (Tumor Imaging), AE monitoring
End of Study	Up to 2 Years	Final Vital Signs, Treatment Discontinuation, AE/SAE Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes 1. Number of participants with Adverse Events (AEs). 2. Number of participants with Serious Adverse Events (SAEs). 3. Number of participants with Dose-Limiting Toxicities (DLTs).

7.2 Secondary Outcomes 1. Maximum observed plasma concentration (Cmax). 2. Time of maximum observed plasma concentration (Tmax). 3. Area under the plasma concentration-time curve (AUC).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring from consent through up to 2 years, specifically focused on combination toxicities. **Serious Adverse Events (SAEs):** Evaluated continuously up to approximately 2 years. **Data Monitoring Committee (DMC):** Safety Review Committee (SRC) oversight utilized to make dose-escalation and cohort expansion decisions.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety set (all participants who receive at least one dose of the investigational product). **Sample Size Justification:** \$N=70\$ targeted participants; sample size driven by standard Phase 1 dose-escalation (e.g., 3+3 design) and expansion cohort requirements to determine the Maximum Tolerated Dose (MTD) or Recommended Phase 2 Dose (RP2D). **Handling of Missing Data:** Missing safety data generally not imputed in Phase 1 trials. Standard descriptive statistics for PK and safety.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: Conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Standard clinical oncology monitoring, including onsite and remote data verification. **Audit Trail:** Verified via an Electronic Data Capture (EDC) system including query resolution tracking for clinical endpoints and AE logging.

11. Study Identifiers & Governance

Primary ID: IM043-004 **Registry Number:** NCT05625412 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** A Study of BMS-986360/CC-90001 Alone and in Combination With Chemotherapy or Nivolumab in Advanced Solid Tumors **Official Regulatory Title:** A Phase 1, Open-label, Multicenter Study of BMS-986360/CC-90001 Alone and in Combination With Chemotherapy or Nivolumab in Advanced Solid Tumors

12. Clinical Framework (Ontology & Disease Specifics)

Disease Areas / Conditions: Advanced Solid Tumors / Advanced Solid Malignancies **Preferred UMLS Name:** Advanced Solid Tumor **Preferred Name:** Advanced Malignant Solid Neoplasm **Semantic Type:** Neoplastic Process **Definition:** A malignant solid neoplasm that has spread extensively to other anatomic sites or is no longer responding to treatment. **SNOMED ID:** 39958004 **SNOMED Hierarchy:** 785317006 | Advanced solid malignant neoplasm | Advanced solid malignant neoplasm (disorder) **Medical Methodology:** Novel Agent combined with Standard Chemotherapy/Immunotherapy (Docetaxel, Capecitabine, or Nivolumab) **Optimization Target:** Maximum Tolerated Dose (MTD) and Recommended Phase 2 Dose (RP2D) Evaluation

13. Study Procedures & Methodology

Management Pathway: Phase 1 Dose Escalation & Expansion **Education Protocol (HCP):** Site Initiation Visits, Investigator Meetings, and Dose Escalation SRC Meetings **Education Protocol (Patient):** Informed Consent, expected AE counseling, and dosing schedule training **Quality Audit Mechanism:** Independent auditing, tumor image central/local review alignment, and PK sample tracking

14. Observation & Follow-Up Schedule

Total Duration: Up to approximately 2 years **Onsite Visit Cadence:** Synchronized with standard oncology treatment cycles (e.g., Q2W, Q3W, or Q4W dosing visits) **Remote Monitoring Frequency:** Depending on safety profile, typically between cycles **Checklist Review Interval:** Regular tumor imaging assessments (typically every 8-12 weeks)

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]				Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]				